



Procedure

Seven Day Post Discharge Follow Up – Mental Health

Original endorsement date	July 2016
Date of current endorsement	April 2025
Version	2.00
Functional subgroup	Mental Health
Summary	This document outlines the process for East Metropolitan Health Service (EMHS) mental health services for appropriately following up with people discharged from inpatient mental health care. Every person discharged from EMHS mental health services will receive a follow up contact.
Procedure owner	Executive Director Mental Health
Contact	
Review date	April 2028
National Standards Mental Health 2010	Delivery of Care, 10.6.8
Impact Statement Declaration Consultation (ISD)	The Aboriginal consultation is reflected in this policy through Aboriginal consultation to ensure cultural appropriateness and positive health impacts on Aboriginal people have been incorporated (ISD Record ID3988).
Status	Active
Information Classification	OFFICAL

1. Background

The immediate post discharge period is a time of marked risk, but rates of suicide remain high for many years after discharge¹. Maintaining safety following discharge from mental health inpatient units requires assertive and coordinated follow-up to address mental health deterioration or suicide risk, with direct contact with the person as soon as possible after discharge. While the national indicators measure follow up in the 7-day period following discharge, the actual timing of follow-up should be determined by the needs of the individual person.

This document outlines the process for East Metropolitan Health Service (EMHS) mental health services for appropriately following up with people discharged from inpatient mental health care. Every person discharged will receive a follow up contact.

Site based policies/procedures will provide direction around roles and responsibilities related to the undertaking of seven day follow up.

Note: Every person who has an overnight admission to a mental health inpatient service, will receive a follow up contact (either face to face or by phone) within 7 days of discharge, including people who do not keep their planned follow up arrangements.

1.1. Key Definitions

Term	Definition
Acute (specialised mental health) inpatient service	WA Health describes an acute (specialised mental health) inpatient unit as a service that provides voluntary and involuntary short-term inpatient management and treatment during an acute phase of mental illness, until the person has recovered enough to be treated effectively and safely in the community. For EMHS these services are: <u>Royal Perth Hospital</u> • Mental Health Emergency Centre, Dabarkan Mental Health Unit, Ward 2K (Bidi Kwop Kaadadjiny) <u>Bentley Health Service</u> • Ward 6, Ward 7, Ward 8, Ward 11, Ward 10A, Ward 10B, Ward 10C • St James Transitional Care Unit (Bidi Wungen Kaat Centre) <u>Armadale Health Service</u> • Moodjar, Banksia, Karri and Yorgum wards • Crisis Resolution Home Treatment Team (Kadadjiny Marr Koodjal Mia)
Ambulatory mental health services	The term ambulatory mental health services is used interchangeably with 'community services' and 'non-admitted mental health services'

¹ [Principles and Best Practice for the Care of People Who May be Suicidal](#), Department of Health, Western Australia

	to describe mental health services provided to people in the community. Ambulatory services are established in PSOLIS as community mental health programs.
Separation (also known as discharge)	A separation for this measure occurs anytime a person leaves an acute mental health service following a planned discharge, or discharge against medical advice.
Psychiatric Services On-Line Information System (PSOLIS)	Mental health clinical information system used by all government mental health agencies throughout Western Australia.
PSOLIS Service Event	Any Service Event is a record of a service to, or on behalf of, a person including one or more of the following: • actual service contact delivered • must involve at least two people, with one being a health professional • Contact can occur face-to-face, via telephone or video. To be compliant as a seven day follow up Service Event, the Service Event must be recorded under an ambulatory program where: • The Community Service Event occurs between one and seven days following Separation/Discharge Date • The Community Service Event Item is a Reportable Event • Client Present or Associate present Flag is marked Y (for yes) in PSOLIS, or, • The Service Contact Category is Triage
Support people	Support people includes carers, family members, nominated people and other people identified by a person accessing care as a personal support person.
Exclusions	Unless specifically requested by the treating Psychiatrist as documented in the health record, the following separations do not require 7 day follow up to occur: • Same day admissions (where the admission and separation dates are the same) • Transfer to another public acute (general health) or mental health unit / other public hospital or Contracted Health Entity • Separation/discharge/transfer where the discharge location is to a community mental health residential service • Separation/discharge where the length of stay is one night only and a procedure code for Electroconvulsive Therapy (ECT) is recorded • Statistical discharges or change of care type discharges • Separations that end in death. For further exclusion information see HSPR Performance Indicator Definition Manual 2024-25. Note: Discharges/transfers to private hospitals are not excluded from 7 day follow up

2.Key Performance Indicators

Seven-day follow-up is measured by the percentage of separations from an acute (specialised mental health) inpatient service for which a community/ambulatory service contact was recorded in the seven days immediately following that separation.

The [Health System Performance Report](#) (HSPR) benchmark rate for seven-day follow up is $\geq 75\%$.

Local compliance is monitored via the [EMHS Mental Health 7 Day follow up P1-1 - Power BI](#). This dashboard allows clinical staff to monitor current discharges and relevant compliant services events, including if the person has been referred to a community mental health service. Access to this dashboard can be provided by contacting the EMHS PSOLIS Area Coordinator.

The [HSPR Performance Indicator Definition Manual 2024-25](#) provides further information on how the seven day follow up indicator is measured (*P1-1 Percentage of post-discharge community care within 7 days following discharge from acute specialised mental health inpatient services*).

3.Pre-Discharge Requirements

Prior to discharge the inpatient service should perform the following:

- Confirm the person's contact details in WEBPAS, both on admission to, and discharge from the inpatient setting.
- The Treatment, Support and Discharge Plan (TSDP) will be collaboratively developed with the person and their identified support people. A copy of the TSDP should be provided to the person and other identified support people involved in their care.
- The Risk Assessment Management Plan (RAMP) should be updated on discharge and specify historic and current risks.
- Every effort will be made to provide a discharge summary within 24 hours of discharge, as per the [EMHS Discharge Communication \(Written\) Policy](#). The Discharge Summary/Care Transfer Summary is used by the General Practitioner and/or other health service provider to support the continued care of the person once they are discharged from hospital.

4.Process

- Seven day follow up must occur in line with the [EMHS Care Coordination in Mental Health Services Policy](#) and site based clinical handover SOPs.
- Seven day follow up can be either completed face to face or via phone with the person or their support people, as indicated by that person's needs.
- All people discharged from a mental health inpatient service, including those who leave against medical advice, should be contacted for a follow-up assessment within seven days of discharge.
- An attempt to contact the person and/or relevant support person needs to occur in a timely manner to allow a minimum of three (3) attempts at contact prior to the end of

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the seven (7) day period. This is to allow the person time to engage and/or care to be escalated if needed.

All attempts to contact, including unsuccessful contacts, must be recorded in the person's medical record in addition to a service event being placed on PSOLIS.

Each person discharged from an inpatient unit is required to have:

- An open referral or activation to a relevant ambulatory program
- Service events entered as POST DISCHARGE FOLLOW UP
- CLIENT PRESENT and/or ASSOCIATE PRESENT indicated in the POST DISCHARGE FOLLOW UP service event, as appropriate.

Any referrals entered for the sole purpose of capturing 7 Day Follow up Activity should be closed after 8 days to ensure data integrity.

Table 1 lists the available relevant service events to be utilised and that will capture compliance measure against 7 day follow up.

Table 1. Available relevant service events

SCENARIO	SERVICE EVENT ITEM <i>(Any reportable service event may be used, these are examples only)</i>	DETAILS
7 Day Follow Up – Person	POST DISCHARGE FOLLOW UP ASSESSMENT	Early contact with the person, within 72 hours of discharge, is preferred. A face-to-face conversation with the person is preferred, however, a verbal conversation over the phone is acceptable.
7 Day Follow Up – Associate	POST DISCHARGE FOLLOW UP FAMILY SUPPORT	Where the person is unable to be contacted following multiple, documented attempts, a verbal conversation with an associate (NOK, carer or nominated personal support person) is appropriate.
7 Day Follow Up – Active Community and Triage	POST DISCHARGE FOLLOW UP ASSESSMENT DEPOT ADMINISTRATION TRIAGE	7-day follow-up assessment does not replace the need for face-to-face follow-up with a community team at the earliest opportunity following discharge from an inpatient setting, for a person already active with a community service. A triage service event may also be recorded and will count as a completed 7-day follow up contact where the person is present.
7 Day Follow Up – No Contact	Various	Where a person might decline clinical follow up or is unable to be contacted, the mental health service should, in collaboration with the referrer and any involved carers, evaluate the risk of the person not receiving follow up assessment, and take appropriate

		action. Refer to EMHS Mental Health Consumers Who Decline Assessment. In instances where the person is unable to be contacted, clinical decision making must also be reflected in the medical record. In circumstances where the person has no fixed abode, the inpatient teams and the community teams are advised to work with the person, prior to discharge to arrange a suitable time and place for follow up.
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5. Escalation of Care

Where the clinician is unable to contact the person and/or relevant support person within three attempts, or earlier if there are clinical concerns, care will be appropriately escalated.

In escalating care, the Clinician responsible for making the follow up call will:

- inform the Inpatient Treating Team Consultant or their delegate at the daily ward handover or Multidisciplinary Team meeting
 - The on-call Consultant Psychiatrist will be the delegate for the Inpatient Treating Team Consultant after hours/ weekends
- inform the person's Care Coordinator and/or relevant Community Mental Health Service (if referred) of unsuccessful phone contact with the person

The urgency of response will be determined based on the person's risk assessment, clinical judgement of the reviewing team and communication between the inpatient team and the community team.

- The Clinician will refer the person to the CTT, ATT, or General Practitioner for follow up as indicated.
- All risk assessments, escalation of care and actions taken to follow up a person will be documented in their medical record.

6. Documentation

All attempts to contact, including unsuccessful contacts, must be recorded in the person's medical record in addition to a service event being placed on PSOLIS.

7. Compliance monitoring and legislative penalties

7.1. Compliance monitoring

Each EMHS Site/Service Executive Director are responsible for ensuring staff in their area/department are made aware of and comply with this policy, and that compliance risks and training requirements are managed.

Staff are to be reminded that compliance with all policies is mandatory.

Local policies/procedures should align with this procedure but may vary regarding responsibility of who is assigned to complete the seven day follow up.

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- The Mental Health Service will monitor compliance with target >75% monthly.
- [HSPR | EMHS - Performance Indicators \(health.wa.gov.au\)](#)
- [EMHS Mental Health 7 Day follow up P1-1 - Power BI](#)

7.2. Legislative obligations

There are legislative obligations in relation to this policy and non-compliance may result in penalties being applied.

8. Legislation

- Health Services Act 2016 section 219, 220, 221
- Public Sector Management Act 1994 section 80A, 80, 81, 82

9. Additional references

9.1. WA Health policies

- MP 011/19 Performance Management Policy–
- MP 0181/24 [Safety-Planning-for-Mental-Health-Consumers-Policy.pdf](#)
- MP 0012/16 [Mental Health Consumers Who Are Missing or Absent Without Leave Policy](#)
- MP 0099/18 [Community Mental Health Status Assessments: Role of Mental Health Clinician Policy](#)

9.2. EMHS policies

- [Discharge Communication](#)
- [Mental Health Consumers Who Decline Assessment](#)
- [Rights of Carers and Other Personal Support Persons](#)

9.3. Other references

- Chief Psychiatrists Standards for Clinical Care
- The Fourth National Mental Health Plan 2009 – 2014
- The Fifth National Mental Health and Suicide Prevention Plan 2017
- WA [DoH Principles and Best Practice for the Care of People Who May Be Suicidal](#)
- Health Strategic Intent 2015-2020
- “Review of the admission or referral to and the discharge and transfer practices of public health facilities and services in Western Australia” Stokes Review 2012